

Northgate Eye Specialists

Encounter: NE-2023-01447

Neuro-Ophthalmology Service · 2210 Northgate Blvd, Suite 300

PATIENT: WHITFIELD, DANA L. DOB: 03/14/1991 MRN: NES-30916
VISIT: FOLLOW-UP CONSULT DATE: 01/20/2023 PROVIDER: C. OKAFOR, MD

INTERVAL HISTORY

Returns six weeks after diagnosis of idiopathic intracranial hypertension (G93.2), confirmed by lumbar puncture on 12/16/2022 with opening pressure of 32 cm H2O. Acetazolamide 500 mg PO BID started 12/16/2022 and has been tolerated aside from mild paresthesias. Depo-Provera (medroxyprogesterone acetate) was discontinued as of 01/05/2023 given the temporal relationship between injectable contraceptive exposure and symptom onset; treating providers agree the agent is best regarded as a contributing factor and should be discontinued permanently.

CURRENT SYMPTOMS

Headaches are markedly improved: two mild episodes in the past month versus daily headaches in November. Denies pulsatile tinnitus. No transient visual obscurations since early January. Reports vision is subjectively back to baseline.

EXAMINATION

VA 20/20 OD, 20/20 OS. Pupils equal, no RAPD. Color plates 14/14 OU. Fundus: optic disc edema improving; Frisen grade 1 OU (previously grade 3 OD, grade 2 OS on 11/21/2022). No hemorrhages.

VISUAL FIELD SUMMARY

	RIGHT EYE (OD)	LEFT EYE (OS)
MD:	-2.8 dB	-2.1 dB
PSD:	2.9 dB	2.4 dB
VFI:	94%	96%
Reliability:	FL 1/13 FP 2% FN 1%	FL 0/13 FP 1% FN 2%

INTERPRETATION

Enlarged blind spot OU, improved from 12/16/2022 baseline (MD -5.4 / -4.6).
No new focal defects. Findings consistent with resolving papilledema.

OCT RETINAL NERVE FIBER LAYER

Average RNFL:	OD 128 um (prev 186)	OS 119 um (prev 171)
Superior:	OD 156 um	OS 149 um
Inferior:	OD 161 um	OS 152 um
Interpretation:	optic disc edema resolving bilaterally.	

ASSESSMENT AND PLAN

1. Idiopathic intracranial hypertension (G93.2) · clinically improving on acetazolamide 500 mg BID. Continue current dose; recheck in 8 weeks.
2. Papilledema, bilateral (H47.10) · Frisen grade 1 OU, improving.
3. Contraception counseling deferred to gynecology; injectable medroxyprogesterone remains contraindicated in this patient.
4. Return precautions reviewed for headache or new visual symptoms.

Electronically signed: C. Okafor, MD · 01/20/2023 15:22